

Health Without a Fixed Address

Sexual and Reproductive Health Access for LGBTQ Migrants and Sex Workers in Transit Along the Kenya-Uganda Border

Trans+

Advocacy, legal support, health access, and digital safety for queer sex workers on the Kenya-Uganda border

This document discusses health care denial, police interference in health access, and violence against LGBTQ people and sex workers. Prepared and published by Trans+.

Executive Summary

Access to sexual and reproductive health care is inseparable from safety for queer sex workers moving through the Kenya-Uganda border region — and both are undermined by the same forces of criminalization and policing described elsewhere in this series. This paper examines how legal risk, stigma, and constant movement combine to push people away from care at precisely the moments they need it most, and describes Trans+'s peer-led model for closing that gap.

1. A Double Exclusion

Queer sex workers in this region are frequently excluded from health systems twice over: as sex workers in general clinical settings, where stigma and, in some cases, mandatory reporting requirements discourage disclosure, and as LGBTQ people in settings designed around sex worker outreach but not around sexual orientation or gender identity. Uganda's Anti-Homosexuality Act, 2023 has made this exclusion more acute by imposing a duty on many professionals, including some health workers, to report suspected same-sex conduct — a provision that international health bodies, including a joint statement from the Global Fund, UNAIDS, and PEPFAR, warned would obstruct HIV prevention and treatment access specifically because it discourages people from seeking care at all.

For people who are also migrants or in transit, a third barrier compounds the first two: the absence of a fixed address or stable clinic relationship. Continuity of care — critical for antiretroviral therapy, PrEP, hormone therapy, and other ongoing treatment — is extremely difficult to maintain when someone is moving between towns, crossing a border, or living in an informal settlement or refugee camp without a consistent provider.

2. Health Risk Is a Safety Issue, Not Just a Medical One

The health risks facing this population cannot be separated from the policing and legal risks described in this series. Fear of arrest deters people from carrying condoms, PrEP, or hormone medication, because these items have in some documented cases been used by police as evidence of sex work or of gender nonconformity to justify a stop or search. Fear of exposure deters people from disclosing their sexual orientation, gender identity, or occupation to health workers, even when that disclosure would lead to better, more relevant care. And the same custodial violence documented in this series' legal support paper — including sexual assault in detention — creates direct, acute health needs, including access to post-exposure prophylaxis and trauma care, that are rarely available inside the custodial system itself.

Within Kenya's refugee camp system, this dynamic is particularly stark. Independent reporting, including from Amnesty International and the Organization for Refuge, Asylum and Migration, has documented high rates of physical and sexual violence against LGBTQ residents of Kakuma refugee camp, alongside significant gaps in how camp health and protection services respond to survivors, particularly where the violence is motivated by anti-LGBTQ hostility rather than being treated as a general protection concern.

3. Specific Health Needs Across the Population

3.1 HIV Prevention and Treatment

Consistent access to HIV prevention tools, including PrEP, and to antiretroviral therapy for those living with HIV, is one of the most continuity-dependent needs in this population. Interruptions caused by border crossings, displacement, or fear-driven avoidance of clinics can have serious health consequences, and restarting treatment after a gap often requires navigating a new clinic relationship from scratch.

3.2 Gender-Affirming Care

Transgender community members face a particularly fragmented landscape, with gender-affirming care — including hormone therapy — inconsistently available across both countries and frequently concentrated in a small number of urban providers that are difficult for someone in transit or in a rural border area to reach. Interruption of hormone therapy carries both physical and psychological health risks.

3.3 General Sexual and Reproductive Health

Contraception, STI testing and treatment, and general reproductive health care are often the most immediately accessible entry point into the health system for this population, but uptake remains limited by the same stigma and disclosure risks described above, particularly in settings where a provider's response to a disclosure of sex work or LGBTQ identity cannot be predicted in advance.

3.4 Trauma and Mental Health

Given documented rates of violence — in custody, in transit, and within camp settings — trauma-informed mental health support is a significant unmet need, with very few providers in the region trained specifically to work with LGBTQ survivors of police or community violence.

COMMUNITY TESTIMONY (IDENTITY WITHHELD FOR SAFETY)

A community health worker described a client who had stopped taking antiretroviral medication for several weeks after being displaced from a border town, because she had no way to safely carry her medication without risking questions from officers about what it was and why she had it.

4. Trans+'s SRHR Model

4.1 Peer-Led Outreach

Trans+'s primary approach to closing this gap is peer-led outreach conducted by community health workers who are themselves current or former sex workers. Because they share lived experience with the people they serve, they are frequently able to reach community members who would not otherwise engage with formal health services, and to provide information and referrals for HIV prevention and treatment, STI testing and treatment, contraception, and general sexual health in a setting the client already trusts.

4.2 A Vetted Provider Network

Rather than relying solely on formal referral pathways, Trans+ maintains an informal, community-recommended network of clinics and individual providers who have demonstrated they can deliver genuinely stigma-free care. This allows outreach workers to make referrals with confidence, based on direct community feedback about how a given provider actually treats LGBTQ and sex worker clients, rather than on formal credentials alone.

4.3 Continuity Support for People in Transit

Recognizing that many clients are moving between locations, Trans+ works to support continuity of care across that movement — including help navigating a change of provider, carrying documentation of an existing treatment plan, and connecting with partner providers on the other side of the border where possible.

4.4 Gender-Affirming Care Navigation

For transgender clients, Trans+ helps navigate the fragmented landscape of gender-affirming care across both countries, connecting people to available providers and, where formal care is not accessible, providing harm-reduction information to reduce the risks associated with unsupervised hormone use.

5. Recommendations

1. **To health ministries in both countries:** Explicitly exempt health workers from any duty-to-report requirements tied to sexual orientation or gender identity, given the documented chilling effect on care-seeking behavior.
2. **To clinic and camp health administrators:** Provide specific training on stigma-free care for LGBTQ sex workers, and establish clear, confidential pathways for survivors of custodial or camp-based sexual violence to access post-exposure prophylaxis and trauma care.
3. **To international health and donor bodies:** Fund continuity-of-care mechanisms specifically designed for mobile and displaced populations, rather than models that assume a stable clinic relationship.
4. **To UNHCR and camp protection services:** Strengthen the connection between protection response and health response for LGBTQ survivors of violence within camp settings, so that a reported hate crime reliably triggers access to appropriate medical and psychological care.

Trans+

This briefing was researched and prepared by Trans+, an organization led by queer and transgender sex workers working along the Kenya-Uganda border. It draws on documentation gathered through Trans+'s health outreach program, alongside publicly available human rights and public health reporting. For more information, partnership inquiries, or to support this work, please contact Trans+ directly.

© Trans+. This document may be shared and cited with attribution for advocacy, journalistic, and educational purposes.